Pembrolizumab every 6 weeks for head and neck cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pembrolizumab monotherapy

Active medicines: Pembrolizumab

Emetogenic risk: Minimal

Incomplete clinician/pharmacy review draft. Individualise and authorise before use.

Additional phase-specific support

- Routine preventive anti-sickness or steroid medicines are not usually needed for this monotherapy. Steroids or hormone replacement may be prescribed for a specific complication; they are not a generic prophylactic prescription.
- Carry your immune alert card during and after pembrolizumab. Tell any clinician that you have received immunotherapy. Do not restart a held treatment or change prescribed steroids or hormone replacement yourself.
- Tell the team early if swallowing, appetite, pain or taste changes reduce intake. A dietitian can agree practical food, drink or feeding support with you. Do not wait for substantial weight loss.
- Agree a mouth-care plan and tell your dentist about previous or current cancer treatment. Arrange oncology advice before dental procedures, particularly after head and neck radiation.
- Ask for speech and language therapy if swallowing or speech is difficult. Use the food textures, exercises and communication support advised for you; a general guide cannot establish safe swallowing.
- Bring prescribed, over-the-counter and herbal medicines and supplements for review. Discuss fertility preservation and breastfeeding before treatment. Use effective contraception during treatment and for at least four months after the final dose. Tell the team promptly about a possible pregnancy. Do not breastfeed without specific advice from the team.
- Keep reporting new symptoms after treatment ends. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00558 v13 · pNCCP treatment, tests and supportive-care sections · NCCP 00558 v13 and current local policy

Sources checked 2026-11-09 · SACTCheck v0.84.46 · Regimen-level drafts; prescriber and oncology-pharmacy verification required